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those who advocate for competition in healthcare care to admit.
2.2. How Stakeholders Can Influence Policy Development
Healthcare policy development is seldom a linear, “top-down” affair. Several actors (royal colleges, trade unions, patient advocacy groups, pharmaceutical companies) try to influence policy to align with their interests, values, and expertise (Alford, 1975). Structural position, access to political elites and ability to secure media coverage all play a part in determining whose voices are heard (WHO, 2021).
Historically, professional organisations like the British Medical Association have enjoyed a significant influence (Ham, 2024). In fact, it took the NHS until 1948 to be formed after the infamous “stuffing the doctors' mouths with gold” by Aneurin Bevan, who let the doctors have their heads, in exchange for their cooperation with the NHS, and continued to allow them to receive private patients (Klein, 2013). The same scenario played out with the 2012 Act. when the Royal College of Nursing and British

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Task 2: Theoretical Frameworks, Concepts and Models in Developing Policy

2.1. Ideological Backgrounds Influencing Healthcare Policy Development

Healthcare policy is very much determined by the ideological orientation of those in power, and any analysis of the reasons for the shape of certain reforms begins with an understanding of those orientations (Tuohy, 2018). An influential structural analysis was provided by Alford (1975) who suggested that there are 3 interests in the healthcare field: the dominant professional monopolisers (doctors), the challenging corporate rationalisers (managers and planners) and the repressed community population (patients and public). This model, which was developed within the American context, has nevertheless important analytical value in Britain, where the medical profession has traditionally had strong influence over health policy (Hollingsworth, Hage and Hanneman, 2019). The period of Labour government between 1997 and 2010 was a social democratic ideological frame: the role of the state was to ensure fairness in access to healthcare, and it increasingly introduced market-liberal tools (Taylor-Gooby et al. 2019). Powell (1997) suggested that the ideology of 'the third way' of New Labour involved an unstable patching of market elements onto a universalist welfare system, which produced tensions within the system. The idea of the NHS founding settlement to be an egalitarian force and the pricing mechanism of payment by results could never be combined satisfactorily.

The Coalition government was more explicit in its use of the neo-liberal assumptions namely that competition brings about improvements in quality, that consumer choice is a better allocative mechanism than professional planning, and that the state should be a commissioner, not a provider (Hunter 2008). Such assumptions influenced the Health and Social Care Act (2012) as regards the expanded role of the market and the diminished role of democratic responsibility for health commissioning. Paton (2014) challenged these assumptions on the empirical basis, as the evidence on the effect of competition on improving outcomes in healthcare is far less robust than those who advocate for competition in healthcare care to admit.

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